PATIENT

Workflow	Steps	Data to be stored	Audit events to log
Register for CareQ Not in Replit	1. System generates an invite token linked to the patient's referral/case. 2. System sends SMS/email invite with a secure link (no PHI). 3. Patient clicks the invite link. 4. System validates token (valid / expired / used). 5. Patient completes identity verification (e.g., DOB + postal code or OTP). 6. System validates identity match (or blocks after too many failed attempts). 7. Patient creates account credentials (email + password) and sets communication preferences. 8. System creates Patient account and links it to the existing case/referral. 9. System sets patient status to `Registered` (or `Registered_Pending_Consent`). 10. System logs registration audit event. 11. System redirects patient to the Consent workflow.	• Invite token ID • Invite token status (created / used / expired) • Linked case/referral ID • Patient user ID • Registration timestamp • Identity verification method (DOB+postal or OTP) • Identity verification result (pass/fail) • Number of verification attempts • Communication preferences (email/SMS/phone) • Email + phone on file at time of registration	• INVITE_SENT • INVITE_LINK_OPENED • TOKEN_INVALID • IDENTITY_VERIFICATION_FAILED • IDENTITY_VERIFICATION_PASSED • ACCOUNT_CREATED • COMMUNICATION_PREFERENCES_SET • REGISTRATION_COMPLETED
Provide consent Not in Replit	1. Patient completes Registration and is routed to the Consent page. 2. System displays the required consent items (virtual care, data use, sharing with AHKC, electronic communication limits, emergency disclaimer). 3. Patient reviews the consent content. 4. Patient checks all required consent boxes. 5. Patient submits consent. 6. System validates that all required consents are accepted. 7. If any required consent is missing, system blocks submission and highlights missing items. 8. If valid, system records consent acceptance. 9. System generates a consent receipt (PDF or record) and stores it. 10. System updates patient status to `Consented` / `Active`. 11. Patient is routed to the Intake Questionnaire workflow.	• Consent version ID (so you know which wording they agreed to) • Consent items accepted (boolean per item) • Consent acceptance timestamp • Consent receipt document ID (or stored file reference) • Patient status after consent (`Consented`)	• CONSENT_PAGE_VIEWED • CONSENT_SUBMITTED • CONSENT_VALIDATION_FAILED • CONSENT_ACCEPTED • CONSENT_RECEIPT_GENERATED • PATIENT_STATUS_UPDATED_TO_CONSENTED
Complete intake questionnaire Not in Replit	1. Patient is routed to Intake after completing Consent. 2. System opens a new Intake form linked to the patient and their case/referral. 3. Patient completes demographics/contact confirmation (as allowed). 4. Patient selects joint + laterality (hip/knee; left/right/both). 5. Patient enters symptom details (duration, severity, function limits). 6. Patient enters prior treatments tried (PT, injections, meds, etc.). 7. Patient completes red-flag screen (safety questions). 8. Patient enters medical history, surgeries, meds, allergies. 9. Patient enters imaging history (type/date/location; upload optional if you support it). 10. Patient submits intake form. 11. System validates completeness of required fields. 12. If incomplete, system shows missing sections and keeps intake in "incomplete" state.	• Intake form ID + patient ID + case/referral ID • Intake form status (incomplete / complete) • All intake responses (structured fields + free text where needed) • Red-flag responses + derived `UrgentFlag` boolean • Intake submission timestamp • Completeness status (required sections complete: yes/no)	• INTAKE_STARTED • INTAKE_SAVED (if you allow save/resume) • INTAKE_SUBMITTED • INTAKE_VALIDATION_FAILED (if applicable) • INTAKE_COMPLETED • RED_FLAG_POSITIVE (if applicable) • URGENT_REVIEW_FLAG_SET (if applicable) • ROUTED_TO_SCHEDULING

	13. If red flags are positive, system flags case as needing urgent review and shows instructions to call clinic/emergency guidance. 14. If complete and no urgent flag, system marks intake complete and routes patient to Scheduling (Book a virtual visit).		
Schedule a virtual visit Not in Replit	1. System determines a visit is needed (after intake completion, follow-up due, provider escalation, or patient requests review). 2. System creates a "Schedule visit" task for the patient. 3. System sends notification (no PHI) prompting patient to book. 4. Patient logs into the platform. 5. Patient opens the scheduling page. 6. Patient selects visit type (initial vs follow-up). 7. Patient selects visit modality (virtual vs VOIP, if applicable). 8. System displays available time slots based on clinic/provider availability. 9. Patient selects a time slot. 10. Patient confirms booking. 11. System creates an Appointment record linked to the case. 12. System sets appointment status to `Scheduled`. 13. System generates a Pre-Visit form linked to this appointment. 14. System schedules reminder and confirmation messages. 15. System shows confirmation to patient in the portal. 16. System sends a calendar invite to patient with link.	• Appointment ID • Patient ID • Case/referral ID • Visit type (initial / follow-up) • Visit modality (virtual / VOIP) • Scheduled date and time • Appointment status (scheduled / cancelled / completed / no-show) • Pre-Visit form ID linked to appointment • Reminder schedule timestamps • Confirmation status (pending / confirmed)	• SCHEDULING_TASK_CREATED • SCHEDULING_PAGE_VIEWED • SLOT_SELECTED • APPOINTMENT_CREATED • APPOINTMENT_CONFIRMED_BY_PATIENT (if confirmation happens at booking) • PREVISIT_CREATED • REMINDERS_SCHEDULED • APPOINTMENT_BOOKING_COMPLETED

Visit reminder	1. System identifies appointments occurring within 48 hours.
	2. System sends first reminder by SMS/email (no PHI) containing three actions:
	  ○ Confirm appointment
	  ○ Cancel appointment
	  ○ Reschedule appointment
	3. Patient selects an action directly from SMS/email reply.
	4. If patient selects **Confirm**, system updates appointment status to `confirmed`.
	5. If patient selects **Cancel**, system updates appointment status to `cancelled_by_patient` and removes appointment from schedule.
	6. If patient selects **Reschedule**, system routes patient to the Reschedule workflow.
	7. If no action is taken, system sends a second reminder at 24 hours before the appointment.
	8. If still no action, system sends a third reminder at 12 hours before the appointment.
	9. If still no response, system triggers an outbound call task for clinic staff between 3–6 hours before the appointment.
	10. Staff calls patient to confirm, cancel, or reschedule.
	11. Staff records the outcome of the call in the system (confirmed / cancelled / reschedule requested / no answer).
	12. If no response after call attempt, appointment remains marked as `unconfirmed`.
	13. Staff dashboard displays appointment as "Unconfirmed – high no-show risk".
		• Appointment ID
		• Confirmation status (`pending / confirmed / unconfirmed`)
		• Cancellation status (if cancelled by patient)
		• Reminder attempt count (1st, 2nd, 3rd sent)
		• Timestamps for each reminder sent
		• Patient action source (SMS link / email link / staff call)
		• Call attempt timestamp (if required)
		• Call outcome (confirmed / cancelled / reschedule requested / no answer)
		• Final appointment state at visit time
			• REMINDER_48H_SENT
			• REMINDER_24H_SENT
			• REMINDER_12H_SENT
			• CONFIRMATION_RECEIVED
			• CANCELLATION_RECEIVED
			• RESCHEDULE_REQUESTED
			• CALL_TASK_CREATED
			• CALL_ATTEMPTED
			• CALL_OUTCOME_RECORDED
			• APPOINTMENT_LEFT_UNCONFIRMED

	14. Appointment can still proceed even if unconfirmed.		
Complete pre-visit history	1. System detects an upcoming appointment that requires pre-visit completion. 2. System creates a Pre-Visit form linked to the appointment. 3. System sends notification (SMS/email, no PHI) prompting patient to complete pre-visit. 4. Patient opens the secure link to the CareQ portal. 5. System displays the Pre-Visit questionnaire specific to that appointment. 6. Patient reports changes since last visit (symptoms better/same/worse). 7. Patient reports any new injuries or acute changes. 8. Patient reports ER visits or hospitalizations since last visit. 9. Patient reports new medications or allergies. 10. Patient reports new imaging or tests completed. 11. Patient completes a short red-flag screen. 12. Patient submits the Pre-Visit form. 13. System validates required fields. 14. If incomplete, system shows missing items and keeps status as `incomplete`. 15. If complete, system marks Pre-Visit status as `complete`. 16. If red-flag responses are positive, system flags the case as `UrgentReviewRequired` and displays instruction to call clinic/emergency guidance. 17. System updates appointment readiness flag to `ready_for_visit = true` when form is complete. 18. System makes Pre-Visit summary available to nurse and GP in provider view.	• Pre-Visit form ID • Appointment ID • Patient ID • Pre-Visit form status (`incomplete / complete`) • All questionnaire responses (structured fields) • Red-flag responses • Derived urgent flag (`true / false`) • Submission timestamp • Last updated timestamp • Appointment readiness flag (`ready_for_visit`)	• PREVISIT_CREATED • PREVISIT_LINK_SENT • PREVISIT_STARTED • PREVISIT_SAVED (if partial save supported) • PREVISIT_SUBMITTED • PREVISIT_VALIDATION_FAILED • PREVISIT_COMPLETED • RED_FLAG_POSITIVE • URGENT_REVIEW_FLAG_SET • PREVISIT_AVAILABLE_TO_PROVIDERS
Receive a visit cancellation	1. Clinic staff identifies that an upcoming appointment must be cancelled (e.g., provider unavailable, clinic issue). 2. Staff opens the Appointment in the system. 3. Staff selects **"Cancel appointment (clinic)"**. 4. System requires staff to select a cancellation reason (e.g., provider unavailable, technical issue, clinic emergency). 5. System updates appointment status to `cancelled_by_clinic`. 6. System removes the appointment from the provider schedule. 7. System automatically creates a new **"Schedule visit" task** for the patient. 8. System sends SMS/email notification to patient (no PHI): ○ "Your CareQ appointment has been cancelled by the clinic. Please book a new time." 9. Patient dashboard updates to show that the appointment is cancelled. 10. Patient sees a prompt to rebook when they next log in. 11. Appointment remains linked to the case for historical tracking.	• Appointment ID • Previous appointment date/time • Appointment status = `cancelled_by_clinic` • Cancellation reason (structured list) • Cancellation timestamp • Staff user ID who cancelled • Rebooking task ID created • Notification sent timestamp • Patient case ID	• CLINIC_CANCELLATION_INITIATED • APPOINTMENT_CANCELLED_BY_CLINIC • CANCELLATION_REASON_RECORDED • REBOOKING_TASK_CREATED • PATIENT_NOTIFIED_OF_CANCELLATION • APPOINTMENT_UPDATED_IN_SCHEDULE
Complete virtual visit	1. Appointment enters start window (e.g., 10–15 minutes before scheduled time). 2. System activates the **Join Visit** button in the patient portal.	• Appointment ID • Visit modality = `virtual`	• VIRTUAL_VISIT_LINK_ACTIVATED • PATIENT_JOINED_MEETING

	3. System generates or retrieves the unique Zoom meeting link for the appointment. 4. Patient clicks **Join Visit** from the CareQ portal. 5. System redirects patient to the Zoom meeting. 6. Zoom waiting room displays on-screen guidance (e.g., “Your CareQ visit will begin shortly. You will first meet the nurse, then the doctor.”). 7. System records **patient entry timestamp**. 8. Virtual clerk joins the Zoom meeting. 9. System records **clerk entry timestamp**. 10. Clerk confirms patient identity (name + DOB). 11. Clerk performs audio/video check. 12. Clerk confirms backup phone number for VOIP fallback. 13. Clerk explains visit flow. 14. Clerk marks “tech check complete” in the system. 15. Clerk exits the Zoom room. 16. System records **clerk exit timestamp**. 17. Nurse joins the Zoom meeting. 18. System records **nurse entry timestamp**. 19. Nurse opens patient chart in CareQ. 20. System displays Pre-Visit summary and highlights missing structured fields. 21. Nurse reviews Pre-Visit responses with patient. 22. Nurse clarifies discrepancies and adds missing clinical details (provider-only). 23. Nurse updates medications and allergies if needed. 24. Nurse confirms imaging and investigation history. 25. Nurse re-screens for red flags. 26. If red flags present, nurse sets `UrgentFlag = true`. 27. System surfaces urgent indicator in GP view. 28. Nurse marks structured intake complete in system. 29. Nurse exits the Zoom room. 30. System records **nurse exit timestamp**. 31. System recalculates risk score, risk category, and computed wait time. 32. System stores updated scoring snapshot linked to this visit. 33. GP joins the Zoom meeting. 34. System records **GP entry timestamp**. 35. GP reviews nurse intake, Pre-Visit summary, prior notes, tasks, and risk outputs. 36. GP addresses safety issues and red flags first. 37. GP reviews risk category and computed wait time. 38. GP accepts or overrides the system-generated risk category. 39. If overridden, GP provides a brief rationale. 40. GP defines the care plan (investigations, referrals, recommendations, follow-up timing, escalation if needed). 41. System converts each plan item into executable **tasks** for appropriate clinic roles. 42. System assigns task owners and due dates. 43. After the plan is finalized, system generates a **draft clinical note** using AI scribe output + structured fields. 44. GP reviews and edits the draft note as needed.	• Zoom meeting ID or link reference • Patient entry timestamp • Patient exit timestamp • Clerk entry timestamp • Clerk exit timestamp • Nurse entry timestamp • Nurse exit timestamp • GP entry timestamp • GP exit timestamp • NurseStructuredIntake data • Red-flag indicator (`UrgentFlag`) • Risk score, risk category, computed wait time • Scoring snapshot timestamp • Visit note ID • Task IDs generated from the plan • Appointment status = `completed` • PatientWantsSummary flag (yes/no)	• PATIENT_LEFT_MEETING • CLERK_JOINED_MEETING • CLERK_LEFT_MEETING • NURSE_JOINED_MEETING • NURSE_LEFT_MEETING • GP_JOINED_MEETING • GP_LEFT_MEETING • TECH_CHECK_COMPLETED • NURSE_INTAKE_COMPLETED • RISK_RECALCULATED • RISK_OVERRIDE_APPLIED (if applicable) • PLAN_FINALIZED • TASKS_CREATED_FROM_PLAN • VISIT_NOTE_SAVED • PATIENT_SUMMARY_PREFERENCE_RECORDED • VISIT_COMPLETED

	45. GP saves the finalized visit note. 46. GP explicitly records whether the patient wants to receive a visit summary and next steps (`PatientWantsSummary = yes/no`). 47. System stores this preference linked to the visit. 48. GP marks the visit as complete. 49. System updates appointment status to `completed`. 50. GP exits the Zoom meeting. 51. System records **GP exit timestamp**. 52. Patient exits the Zoom meeting. 53. System records **patient exit timestamp**.		
Technical difficulties	1. Technical issue occurs during a scheduled virtual visit (e.g., audio failure, video failure, dropped connection, platform crash). 2. Patient or staff verbally reports the issue during the call, or staff observes loss of connection. 3. Staff attempts basic troubleshooting (refresh, rejoin, device check). 4. If troubleshooting fails within a short window (e.g., 2–3 minutes), staff decides to convert the visit to VOIP. 5. Clerk, nurse, or GP informs patient: "We'll continue this visit by phone." 6. Staff initiates outbound phone call to the patient's confirmed phone number. 7. Patient answers and confirms identity (name + DOB). 8. Visit continues by phone using the same clinical workflow (nurse intake → GP review → plan). 9. Staff marks the appointment as **"Converted to VOIP due to technical issues."** 10. System records the time the conversion occurred. 11. Staff completes the remainder of the visit and marks it complete as usual. 12. If patient cannot be reached by phone, staff marks the visit as **"Failed due to technical issues"** and routes to rescheduling workflow.	• Appointment ID • Original modality = `virtual` • Technical issue flag (`true/false`) • Technical issue type (audio/video/platform/disconnect/unknown) • Time technical issue identified • ConvertedToVOIP flag (`true/false`) • Conversion timestamp • Phone call attempt timestamp • Call outcome (connected / no answer / voicemail) • Final visit modality (`virtual` or `VOIP`) • Visit completion status (`completed` / `reschedule_required`)	• TECHNICAL_ISSUE_REPORTED • TROUBLESHOOTING_ATTEMPTED • VISIT_CONVERTED_TO_VOIP • OUTBOUND_CALL_ATTEMPTED • CALL_CONNECTED • CALL_FAILED • VISIT_CONTINUED_BY_PHONE • VISIT_FAILED_DUE_TO_TECHNICAL_ISSUES • VISIT_ROUTED_TO_RESCHEDULING
Complete VOIP visit	1. A VOIP visit is scheduled directly **or** the virtual visit has been converted to VOIP due to technical issues. 2. System displays the appointment to staff as modality = `VOIP`. 3. At the scheduled time, clinic staff initiates an outbound phone call to the patient's confirmed phone number. 4. Patient answers the call. 5. Staff confirms patient identity (name + DOB). 6. Clerk completes brief orientation (if not already done) and confirms callback number. 7. Clerk hands off the call to the nurse. 8. Nurse conducts structured intake using the provider view in CareQ. 9. Nurse reviews Pre-Visit summary with patient. 10. Nurse clarifies inconsistencies and adds missing clinical details. 11. Nurse updates medications and allergies if required.	1. Appointment ID 2. Visit modality = `VOIP` 3. Outbound call timestamp 4. Call connected flag 5. NurseStructuredIntake data 6. Red-flag indicator (`UrgentFlag`) 7. Risk score, risk category, computed wait time 8. Scoring snapshot timestamp 9. Visit note ID 10. Task IDs generated from plan 11. Appointment status = `completed` 12. PatientWantsSummary flag (yes/no)	• VOIP_CALL_INITIATED • PATIENT_IDENTITY_CONFIRMED • NURSE_INTAKE_COMPLETED • RISK_RECALCULATED • RISK_OVERRIDE_APPLIED (if applicable) • PLAN_FINALIZED • TASKS_CREATED_FROM_PLAN • VISIT_NOTE_SAVED • PATIENT_SUMMARY_PREFERENCE_RECORDED • VOIP_VISIT_COMPLETED

	12. Nurse confirms imaging/investigation history. 13. Nurse re-screens for red flags. 14. If red flags present, nurse sets `UrgentFlag = true`. 15. Nurse marks structured intake complete. 16. Nurse hands off the call to the GP. 17. System recalculates risk score, risk category, and computed wait time. 18. GP reviews nurse intake, Pre-Visit summary, risk outputs, prior notes, and tasks. 19. GP addresses safety issues and red flags first. 20. GP reviews and accepts or overrides risk category (with rationale if overridden). 21. GP defines the care plan (investigations, referrals, recommendations, follow-up timing, escalation). 22. System converts each plan item into executable tasks for appropriate clinic roles. 23. System assigns task owners and due dates. 24. System generates draft clinical note (AI scribe + structured fields) after plan is finalized. 25. GP reviews and edits the note. 26. GP saves the finalized visit note. 27. GP records whether patient wants to receive visit summary and next steps (`PatientWantsSummary = yes/no`). 28. GP marks visit as complete. 29. System updates appointment status to `completed`. 30. Call ends. 31. System records visit end timestamp.	13. Visit start timestamp 14. Visit end timestamp	
No-show to visit	1. Appointment reaches scheduled start time. 2. System checks whether patient has joined the Zoom visit (for virtual) or answered the phone (for VOIP). 3. If patient has not joined/answered within a defined grace period (e.g., 5–10 minutes), system flags the appointment as **"potential no-show."** 4. Clerk attempts to contact patient by phone during the appointment window. 5. If patient answers, clerk confirms identity and: ○ joins them into Zoom, **or** ○ continues the visit by phone (VOIP). 6. If patient does not answer, clerk records call outcome as **no answer / voicemail / number invalid.** 7. If patient successfully joins after outreach, appointment continues normally and is **not** marked as no-show. 8. If patient cannot be reached and does not join, system marks appointment status as `no_show`. 9. System increments patient no-show count. 10. System applies business rules based on count (e.g., warning after first, fee warning after second, termination risk after third) 11. System sends SMS/email notification to patient (no PHI): ● "You missed your CareQ appointment. Please log in to review next steps." 12. System creates a staff task if follow-up is required (e.g., review termination eligibility,	● Appointment ID ● Appointment status = `no_show` ● Grace period duration used ● Clerk outreach attempt timestamp ● Call attempt outcome (no answer / voicemail / invalid number) ● No-show count for patient (running total) ● Policy state (e.g., warning issued, fee warning issued, termination eligible) ● Notification sent timestamp ● Follow-up task ID (if created)	● APPOINTMENT_START_TIME_REACHED ● PATIENT_NOT_PRESENT ● OUTREACH_CALL_ATTEMPTED ● CALL_NO_ANSWER ● APPOINTMENT_MARKED_NO_SHOW ● NO_SHOW_COUNT_UPDATED ● NO_SHOW_NOTIFICATION_SENT ● FOLLOWUP_TASK_CREATED ● NO_SHOW_POLICY_TRIGGERED

	rebooking policy). 13. Appointment remains linked to case for historical tracking.		
Receive post-visit information	1. A visit has been completed and appointment status is `completed`. 2. System checks the flag recorded by GP: `PatientWantsSummary = yes/no`. 3. If `PatientWantsSummary = no`, no post-visit content is released to the patient. 4. If `PatientWantsSummary = yes`, system starts a 24-hour delay timer. 5. During the 24-hour window, staff can still edit/update the finalized note if needed. 6. After 24 hours, system generates the patient-facing package (if not already generated), consisting of: ○ Visit summary ○ Care plan / next steps ○ Patient tasks ○ Any available results that are approved for release 7. System publishes the post-visit package to the patient portal. 8. System sends SMS/email notification (no PHI): ○ "Your CareQ visit summary and next steps are now available in your portal." 9. Patient logs in and views the post-visit information. 10. Patient can download the documents from the portal.	• Appointment ID • Patient ID • PatientWantsSummary flag (already captured at visit) • Visit completion timestamp • Post-visit release eligible timestamp (visit time + 24h) • Post-visit package document IDs (summary, plan, etc.) • Post-visit published timestamp • Notification sent timestamp • Patient view/download timestamps (optional but useful)	• VISIT_COMPLETED_CONFIRMED • POST_VISIT_RELEASE_SCHEDULED • POST_VISIT_RELEASED_TO_PATIENT • POST_VISIT_NOTIFICATION_SENT • POST_VISIT_VIEWED_BY_PATIENT • POST_VISIT_DOWNLOADED_BY_PATIENT
Receive periodic updates	1. System identifies patients eligible for a periodic update based on rules (e.g., every 3 months, waitlist change, task overdue, new result). 2. System generates an internal update event (e.g., "3-month surveillance due", "Waitlist status updated", "No activity in 90 days"). 3. System composes a generic notification (no PHI) appropriate to the trigger. 4. System sends SMS/email notification to patient: ○ "You have a new update in your CareQ portal. Please log in to review." 5. System publishes the relevant update inside the patient portal (e.g., updated status, new tasks, new timeline entry). 6. Patient logs into the portal. 7. Patient views the updated information (status, timeline, task, or message-like card). 8. If the update requires action (e.g., schedule a visit, complete a form), system highlights the required task. 9. System continues monitoring the patient for the next periodic trigger.	• Patient ID • Update type (surveillance reminder / status change / inactivity check / admin update) • Trigger rule that caused the update • Update content ID (what was shown in the portal) • Timestamp update created • Timestamp notification sent • Timestamp patient viewed the update (optional but useful) • Whether update required action (yes/no) • Linked task ID (if created)	• PERIODIC_UPDATE_TRIGGERED • UPDATE_PUBLISHED_TO_PORTAL • UPDATE_NOTIFICATION_SENT • UPDATE_VIEWED_BY_PATIENT • TASK_CREATED_FROM_UPDATE (if applicable)

Log in to platform	1. Patient navigates to the CareQ login page (web or mobile). 2. Patient enters email (or phone) and password. 3. Patient clicks **Log in**. 4. System validates credentials. 5. If credentials are incorrect, system displays an error message. 6. System allows a limited number of failed attempts before temporary lockout (optional but recommended). 7. If credentials are correct, system creates an authenticated session. 8. System routes patient to their dashboard. 9. System displays key dashboard elements (notifications, tasks, appointments, status). 10. System keeps the user logged in until logout or session timeout.	● Patient user ID ● Login timestamp ● Logout timestamp (if captured) ● Session ID ● Session expiry time ● Failed login attempt count ● Account lock status (if implemented) ● Last successful login timestamp	● LOGIN_PAGE_VIEWED ● LOGIN_ATTEMPTED ● LOGIN_FAILED ● LOGIN_SUCCEEDED ● ACCOUNT_LOCKED (if applicable) ● LOGOUT ● SESSION_EXPIRED
Account recovery	1. Patient selects **"Forgot password"** on the login page. 2. System prompts patient to enter their email (or phone number). 3. Patient submits their email/phone. 4. System validates that the account exists. 5. If no matching account exists, system displays a generic error ("If an account exists, you will receive a link"). 6. System generates a secure, time-limited reset token. 7. System sends SMS/email with a password reset link (no PHI). 8. Patient clicks the reset link. 9. System validates the reset token (valid / expired / already used). 10. Patient enters a new password and confirms it. 11. System validates password strength. 12. System updates the account with the new password. 13. System invalidates all existing sessions. 14. System displays confirmation that password has been reset. 15. Patient can now log in with new credentials.	● Patient user ID ● Password reset token ID ● Token creation timestamp ● Token expiry timestamp ● Token status (valid / used / expired) ● Password reset completion timestamp ● Failed reset attempt count (optional) ● Session invalidation timestamp	● PASSWORD_RESET_REQUESTED ● PASSWORD_RESET_LINK_SENT ● PASSWORD_RESET_LINK_OPENED ● RESET_TOKEN_INVALID ● PASSWORD_RESET_FAILED ● PASSWORD_RESET_COMPLETED ● SESSIONS_INVALIDATED
View notifications	1. System generates notifications for the patient based on events (e.g., appointment booked, reminder sent, task created, post-visit info released, update available). 2. System displays a notifications icon or section in the patient dashboard. 3. Patient opens the notifications view. 4. System retrieves all notifications linked to the patient, ordered by most recent first. 5. Unread notifications are visually distinguished (e.g., bold, badge count). 6. Patient selects a notification. 7. System routes patient to the relevant section (e.g., appointment page, task page, document page, status page). 8. System marks the notification as "read." 9. Patient can scroll through past notifications. 10. System retains notifications according to retention policy.	● Notification ID ● Patient ID ● Notification type (appointment, task, result, update, admin, etc.) ● Notification title ● Notification body (portal-safe text, no PHI if externally triggered) ● Linked object (appointment ID, task ID, document ID, etc.) ● Read/unread status ● Created timestamp ● Read timestamp (if read)	● NOTIFICATION_CREATED ● NOTIFICATION_DISPLAYED ● NOTIFICATION_OPENED ● NOTIFICATION_MARKED_READ

View tasks	1. System creates tasks for the patient when required (e.g., complete pre-visit, schedule visit, attend imaging, review documents). 2. Patient navigates to the **Tasks** section of the CareQ platform. 3. System retrieves all active tasks linked to the patient. 4. Tasks are displayed with key attributes (title, brief description, due date, status). 5. Overdue tasks are visually flagged. 6. Patient selects a task. 7. System displays task details and any required action. 8. If the task requires patient action (e.g., complete form, schedule visit), system provides the appropriate link or button. 9. Patient completes the action associated with the task. 10. System updates the task status to `completed` (or `in progress` if partial). 11. Completed tasks move to a "completed" or archived view. 12. System continues to display new tasks as they are created.	• Task ID • Patient ID • Task type (pre-visit, schedule visit, attend test, review document, admin, etc.) • Task title and description • Task owner (patient vs clinic role) • Task status (`pending / in_progress / completed / overdue`) • Due date • Created timestamp • Completed timestamp (if completed) • Linked object (appointment ID, form ID, document ID, etc.)	• TASK_CREATED • TASK_VIEWED • TASK_OPENED • TASK_COMPLETED • TASK_OVERDUE • TASK_STATUS_UPDATED
View results	1. A new result becomes available in the system (e.g., lab report, imaging report). 2. Result is reviewed and marked as **approved for patient release** by a clinician (or released automatically based on rule, if you later support that). 3. System links the result to the patient's case and stores it in the patient results area. 4. System creates a notification for the patient indicating a new result is available. 5. System sends SMS/email (no PHI): "You have a new result available in your CareQ portal." 6. Patient navigates to the **Results** section in the portal. 7. System displays a list of available results (most recent first). 8. Patient selects a result. 9. System displays the result details (e.g., report text, date, ordering context). 10. System provides a download option for the result (PDF or similar). 11. Patient may return to the results list to view other results. 12. System tracks that the result has been viewed (optional but useful).	1. Result ID 2. Patient ID 3. Case/referral ID 4. Result type (lab / imaging / other) 5. Result document or structured data reference 6. Result date 7. Reviewed/released flag (true/false) 8. Release timestamp 9. Notification sent timestamp 10. Viewed timestamp (optional) 11. Downloaded timestamp (optional)	• RESULT_RECEIVED • RESULT_REVIEWED • RESULT_RELEASED_TO_PATIENT • RESULT_NOTIFICATION_SENT • RESULT_VIEWED_BY_PATIENT • RESULT_DOWNLOADED_BY_PATIENT
View past visit summaries	1. One or more visits have been completed and post-visit information has been released to the patient. 2. Patient navigates to the **Visit Summaries** (or "Documents / Visits") section in the portal. 3. System retrieves all released visit summaries linked to the patient. 4. Summaries are displayed in a list ordered by most recent first (with date and visit type). 5. Patient selects a specific visit summary. 6. System displays the full patient-facing summary (plan, next steps, tasks, recommendations). 7. Patient can scroll through the summary. 8. System provides a **Download** option (PDF). 9. Patient can return to the list and view older summaries. 10. System does not display summaries that were not approved for release.	1. Summary document ID 2. Patient ID 3. Appointment ID (linked visit) 4. Summary creation timestamp 5. Summary release timestamp 6. Summary document/file reference 7. Released flag (true/false) 8. Viewed timestamp (optional) 9. Downloaded timestamp (optional)	• VISIT_SUMMARY_RELEASED • VISIT_SUMMARY_VIEWED • VISIT_SUMMARY_DOWNLOADED

View upcoming appointments	1. Patient navigates to the **Appointments** section of the CareQ portal. 2. System retrieves all upcoming appointments linked to the patient. 3. Upcoming appointments are displayed with key details (date, time, modality, status). 4. Appointment confirmation status is clearly shown (confirmed / pending / unconfirmed). 5. Patient selects an appointment. 6. System displays appointment details and available actions. 7. If appointment is within the confirmation window, system displays options: ○ Confirm ○ Cancel ○ Reschedule 8. Patient selects **Confirm**. 9. System updates the appointment confirmation status to `confirmed`. 10. System updates the appointment display to show "Confirmed." 11. If patient selects **Cancel**, system updates appointment status to `cancelled_by_patient` and removes it from upcoming list. 12. If patient selects **Reschedule**, system routes patient to the Reschedule workflow. 13. System reflects all changes immediately in the dashboard. 14. System retains cancelled appointments in history for recordkeeping.	1. Appointment ID 2. Patient ID 3. Appointment date and time 4. Appointment modality (virtual / VOIP) 5. Appointment status (`scheduled / cancelled_by_patient / cancelled_by_clinic / completed / no_show`) 6. Confirmation status (`pending / confirmed / unconfirmed`) 7. Confirmation timestamp (if confirmed) 8. Cancellation timestamp (if cancelled) 9. Reschedule timestamp (if rescheduled) 10. Last updated timestamp	• APPOINTMENT_LIST_VIEWED • APPOINTMENT_DETAILS_VIEWED • APPOINTMENT_CONFIRMED • APPOINTMENT_CANCELLED_BY_PATIENT • RESCHEDULE_SELECTED_FROM_APPOINTMENT • APPOINTMENT_UPDATED_IN_PORTAL
View waitlist status, patient summary and progress and timeline view	1. Patient navigates to the **My Status** (or similar) section of the CareQ portal. 2. System retrieves the patient's current case data. 3. System displays a **Patient Summary** including: ○ Primary issue (e.g., Right knee osteoarthritis) ○ Current pathway stage (e.g., Active surveillance, Awaiting investigations, Sent to surgeon, etc.) ○ Last visit date ○ Next expected action (e.g., "Schedule next follow-up", "Awaiting imaging", "Awaiting booking clerk call") 4. System displays **Waitlist Status**, such as: ○ Current phase (e.g., Under review, In surveillance, Escalated, Referred to surgeon) ○ Priority category (if you choose to expose it) ○ Estimated wait time or general timeframe (if enabled) 5. System displays a **Progress Indicator** (simple visual bar or steps), showing where the patient sits along the journey. 6. System displays a **Timeline** of key milestones, for example: ○ Referral received ○ Registered on CareQ ○ Intake completed ○ First visit completed ○ Follow-up visit completed ○ Investigations ordered ○ Escalated to surgeon	1. Patient ID 2. Case/referral ID 3. Current pathway status (e.g., surveillance, awaiting tests, escalated, sent to surgeon, discharged) 4. Patient summary text (patient-safe) 5. Current phase label 6. Estimated wait time (if shown) 7. Timeline events (event type + timestamp) 8. Last updated timestamp 9. Linked tasks (if any outstanding) 10. Whether this view was accessed (optional analytics)	• STATUS_PAGE_VIEWED • PATIENT_SUMMARY_DISPLAYED • WAITLIST_STATUS_DISPLAYED • TIMELINE_VIEWED • STATUS_UPDATED • PATHWAY_PROGRESS_UPDATED

	○ Sent to booking clerk 7. Timeline entries are ordered chronologically with date labels. 8. Patient can scroll through past milestones. 9. System does **not** display internal-only clinical logic (e.g., raw risk scores, staff comments, override rationale). 10. If the status changes (e.g., escalated, progressed), system updates this view automatically. 11. If the status change requires patient action (e.g., schedule a visit), system highlights the associated task. 12. Patient can return to this view anytime to understand their current situation.		
View resource library	1. Patient navigates to the Resources (or "Learn") section of the CareQ portal. 2. System retrieves the list of available resources. 3. Resources are displayed in organized categories (e.g., Understanding OA, Preparing for surgery, Pain management, What to expect, Exercise & mobility). 4. Each resource shows a title and brief description. 5. Patient selects a resource. 6. System displays the resource content (article, PDF, video, or external link). 7. Patient can scroll, read, or view the content. 8. Patient can return to the resource list to explore more items. 9. System may optionally highlight recommended resources based on patient pathway stage. 10. Resource content does not replace clinical advice and includes a brief disclaimer.	1. Resource ID 2. Resource title 3. Resource category 4. Resource type (article / PDF / video / external link) 5. Resource content or file/link reference 6. Published status (active / inactive) 7. Created timestamp 8. Last updated timestamp 9. Patient view timestamp (optional analytics)	● RESOURCE_LIBRARY_VIEWED ● RESOURCE_OPENED ● RESOURCE_VIEWED ● RESOURCE_DOWNLOADED (if applicable)
View contact information for other services, clinics	1. Patient navigates to the Contacts & Services (or "Help / Directory") section of the CareQ portal. 2. System retrieves the directory content configured by the clinic. 3. System displays a CareQ Contacts section, such as: ○ CareQ general phone number ○ CareQ email (if provided) ○ Office hours ○ Emergency disclaimer ("If urgent, call 911 / go to ER") 4. System displays a Your Clinics & Providers section, such as: ○ Referring clinic name ○ Referred specialty clinic (e.g., AHKC) ○ Booking clinic (if known) 5. System displays an Other Useful Services section, such as: ○ Physiotherapy clinics ○ Imaging centres ○ Injection clinics	● Directory entry ID ● Entry type (CareQ contact / clinic / service / resource) ● Name of organization or service ● Description (short, patient-safe) ● Phone number ● Email (if provided) ● Website URL (if provided) ● Category (e.g., physio, imaging, admin, referral) ● Active/inactive status ● Created / last updated timestamps	● DIRECTORY_VIEWED ● DIRECTORY_ENTRY_OPENED ● EXTERNAL_LINK_CLICKED (optional)

	○ Community resources ○ Government health resources 6. Each entry shows: ○ Name ○ Type of service ○ Phone number ○ Website (if applicable) ○ Location (optional) 7. Patient can scroll through the directory. 8. Patient can tap/click phone numbers to call from their device. 9. Patient can open linked websites in a new tab. 10. System does not imply endorsement unless explicitly stated by clinic. **11.** Directory content is informational only and includes a short disclaimer if needed.		
Download content	1. Patient views a page that contains downloadable content (e.g., result, visit summary, consent, record). 2. System displays a Download button next to each eligible item. 3. Patient selects Download for a specific item. 4. System generates or retrieves a downloadable file (PDF or similar). 5. System initiates the download to the patient's device. 6. Patient may repeat this for multiple individual documents. 7. Patient navigates to a Download Records or My Documents area (if offered). 8. Patient selects "Download all available documents" (optional bundle feature). 9. System gathers all eligible documents (e.g., summaries, results, consents). 10. System generates a single bundled file (e.g., ZIP or combined PDF). 11. System notifies the patient when the bundle is ready (if generation is not instant). 12. Patient downloads the bundled file. 13. System ensures only documents approved for patient access are included in any download. 14. System does not include internal notes, provider-only fields, or unreleased content.	• Document ID • Patient ID • Document type (summary, result, consent, record, etc.) • File reference (PDF, etc.) • Download request timestamp • Download completion timestamp (optional) • Bundle request ID (if bulk download used) • Bundle generation timestamp (if applicable) • Bundle file reference (if applicable) • Release/visibility flag (ensures only approved docs are downloadable)	• DOCUMENT_DOWNLOAD_REQUESTED • DOCUMENT_DOWNLOADED • BUNDLE_DOWNLOAD_REQUESTED • BUNDLE_GENERATED • BUNDLE_DOWNLOADED • UNAUTHORIZED_DOCUMENT_ACCESS_BLOCKED (if attempted)
Report a change in status	1. Patient navigates to the Report a Change (or "Request Review / Update") section of the portal. 2. System displays a short explanation: ○ "Use this if something about your symptoms or situation has changed and you would like the clinic to review." 3. System presents a structured form with fields such as: ○ Reason for update (dropdown: worsening symptoms, new symptoms, new test done elsewhere, change in mobility, other) ○ Free-text description ○ Optional file upload (e.g., external report, letter, image) 4. Patient completes the form.	• Submission ID • Patient ID • Case/referral ID • Reason category • Patient free-text description • Uploaded file references (if any) • Submission timestamp • Review status (pending / reviewed / actioned) • Linked task ID • Task priority •	• STATUS_UPDATE_SUBMITTED • PATIENT_FILE_UPLOADED (if applicable) • REVIEW_TASK_CREATED • STATUS_UPDATE_REVIEWED • ACTION_TRIGGERED_FROM_UPDATE (if applicable)

	5. Patient submits the update. 6. System validates the submission (required fields completed, file size/type acceptable). 7. System stores the submission linked to the patient's case. 8. System creates an internal task for clinic staff (e.g., "Review patient-reported change"). 9. System sets task priority based on reason selected (e.g., higher priority if "worsening symptoms"). 10. System displays confirmation to patient: ● "Your update has been received and will be reviewed." 11. System does not send clinical advice back via messaging. 12. If the review determines a visit is required, staff initiate scheduling via the existing workflow.		
Update profile	1. Patient navigates to Profile or Account Settings in the portal. 2. System displays current profile fields (read-only and editable clearly labeled). 3. Patient edits one or more editable fields, such as: ○ Phone number ○ Email address ○ Preferred name ○ Communication preference (SMS / email / both) 4. Patient selects Save changes. 5. System validates the inputs (e.g., valid email format, valid phone number). 6. If email or phone number is changed, system may trigger a verification step (recommended): ○ Send verification code or link to the new contact method. 7. Patient completes verification (if required). 8. System updates the patient profile. 9. System confirms that changes were saved successfully. 10. Updated contact information is used for all future reminders and notifications.	● Patient ID ● First name / preferred name ● Last name ● Email address ● Phone number ● Communication preference (email/SMS/both) ● Verification status for email ● Verification status for phone ● Profile last updated timestamp ● Previous contact values (optional but useful for audit/history) ●	● PROFILE_VIEWED ● PROFILE_EDIT_STARTED ● PROFILE_UPDATED ● EMAIL_UPDATED ● PHONE_UPDATED ● COMMUNICATION_PREFERENCE_UPDATED ● CONTACT_VERIFICATION_SENT ● CONTACT_VERIFICATION_COMPLETED
Account security	1. Patient navigates to Account Security within Profile/Settings. 2. System displays security options such as: ○ Change password ○ View active sessions (optional) ○ Log out of all devices (optional) 3. Patient selects Change password. 4. System prompts patient to enter current password. 5. Patient enters current password. 6. System validates the current password. 7. Patient enters a new password and confirms it. 8. System validates password strength requirements. 9. Patient selects Save. 10. System updates the password.	● Patient ID ● Password last updated timestamp ● Password reset count (optional) ● Failed password change attempts (optional) ● Session IDs (if session management enabled) ● Session creation timestamps ● Session last active timestamps ● Session invalidation timestamps ●	● ACCOUNT_SECURITY_VIEWED ● PASSWORD_CHANGE_ATTEMPTED ● PASSWORD_CHANGE_FAILED ● PASSWORD_CHANGED_SUCCESSFULLY ● ALL_SESSIONS_INVALIDATED ● SESSION_TERMINATED

	11. System invalidates all existing sessions (recommended). 12. System displays confirmation: “Your password has been updated.”		

View audit log	1. Patient navigates to Privacy & Activity, Audit Log, or similar section in the portal.
	2. System displays a short explanation:
	  ○ “This shows activity related to access and changes to your CareQ records.”
	3. System retrieves audit events associated with the patient’s account and records.
	4. System displays a list of audit entries including:
	  ○ Date/time
	  ○ Event type (e.g., “Visit summary viewed”, “Profile updated”, “Document downloaded”)
	  ○ Actor type (you / clinic staff / system)
	  ○ Description (plain-language, patient-safe)
	5. Patient can scroll through the log chronologically (most recent first).
	6. Patient can filter by category (optional but helpful):
	  ○ Account activity
	  ○ Documents
	  ○ Appointments
	  ○ Data access
	7. Patient selects Download audit log.
	8. System generates a downloadable file (PDF or CSV).
	9. Patient downloads the file to their device.
	10. System confirms successful download.
		• Audit event ID
		• Patient ID
		• Event type
		• Event timestamp
		• Actor type (`patient / staff / system`)
		• Actor ID (internal use only)
		• Patient-safe description
		• Linked object (document ID, appointment ID, etc.)
		• Whether event is visible to patient (true/false)
		• Download request timestamp (if downloaded)
			• AUDIT_LOG_VIEWED
			• AUDIT_LOG_FILTER_APPLIED (optional)
			• AUDIT_LOG_DOWNLOAD_REQUESTED
			• AUDIT_LOG_DOWNLOADED

CareQ initiates termination	1. A termination trigger occurs (e.g., third no-show, prolonged non-engagement, patient unreachable, policy breach, clinical inappropriateness, administrative decision).
	2. Staff member opens the patient’s case in the CareQ system.
	3. Staff selects “Initiate Termination”.
	4. System requires selection of a termination reason from a structured list:
	  ○ Repeated no-shows
	  ○ Unable to reach patient
	  ○ Patient no longer appropriate for pathway
	  ○ Administrative decision
	  ○ Duplicate referral
	  ○ Other (requires free text)
	5. System requires staff to enter a short justification note (internal only).
	6. System displays a confirmation screen explaining impact (case closure, removal from workflows).
	7. Staff confirms termination.
	8. System updates case status to `terminated_by_clinic`.
	9. System immediately:
	  ○ Stops reminders and surveillance
		• Patient ID
		• Case/referral ID
		• Termination status = `terminated_by_clinic`
		• Termination timestamp
		• Termination reason (structured)
		• Internal justification note
		• Staff user ID who initiated termination
		• Confirmation timestamp
		• Outstanding tasks closed (list or flag)
		• Notification sent timestamp
		•
			• TERMINATION_INITIATED_BY_CLINIC
			• TERMINATION_REASON_SELECTED
			• TERMINATION_JUSTIFICATION_RECORDED
			• TERMINATION_CONFIRMED
			• CASE_STATUS_UPDATED_TO_TERMINATED
			• PATIENT_NOTIFICATION_SENT
			• WORKFLOWS_HALTED
			• TASKS_AUTO_CLOSED

	○ Closes outstanding tasks ○ Removes patient from scheduling pools 10. System creates a permanent timeline event: "Case closed by clinic." 11. System sends patient a neutral notification (no PHI): ● "Your CareQ involvement has been closed. Please contact your referring clinic if you have questions." 12. System generates an internal alert/task for admin review (optional but recommended for governance). 13. Patient retains read-only access to previously released documents (unless policy dictates otherwise).		
Patient initiates termination	1. Patient navigates to Settings, My Care, or a similar section in the portal. 2. Patient selects "Withdraw from CareQ" or "Leave waitlist / discontinue CareQ involvement." 3. System displays a clear explanation: ○ What it means (they will no longer be followed by CareQ) ○ That their referral/waitlist status with external clinics may be affected ○ That they can re-enter only via new referral (if applicable) 4. System asks the patient to confirm their decision. 5. System optionally asks for a reason (dropdown + optional free text): ○ No longer interested ○ Pursuing private care ○ Moved ○ Symptoms improved ○ Other 6. Patient confirms termination. **7.** System updates patient case status to `terminated_by_patient`. 8. System removes the patient from active workflows (no more reminders, surveillance, tasks). 9. System generates an internal notification/task for staff: "Patient withdrew from CareQ." 10. System sends a neutral confirmation email/SMS (no PHI): ● "Your CareQ participation has been closed. If you wish to re-enter in the future, contact your referring clinic." 11. Patient retains read-only access to previously released documents 12. System records the termination as a permanent event in the case timeline.	● Patient ID ● Case/referral ID ● Termination status = `terminated_by_patient` ● Termination timestamp ● Termination reason (if collected) ● Free-text explanation (if provided) ● Staff notification task ID ● Last active date ● Whether portal access remains enabled (true/false) ●	● TERMINATION_INITIATED_BY_PATIENT ● TERMINATION_REASON_RECORDED ● TERMINATION_CONFIRMED ● PATIENT_STATUS_UPDATED_TO_TERMINATED ● TERMINATION_NOTIFICATION_SENT ● ACTIVE_WORKFLOWS_STOPPED
Provider initiated escalation	1. New clinical information becomes available (e.g., lab result, imaging report, uploaded document, staff note). 2. System links the new information to the patient's case. 3. System flags the item as "requires clinician review" (manually or via rule). 4. A clinician (GP or designated reviewer) opens the patient's chart.	● Patient ID ● Case/referral ID ● Escalation ID ● Trigger source (result / document / review / other)	● NEW_CLINICAL_INFORMATION_RECEIVED ● INFORMATION_REVIEWED_BY_CLINICIAN ● ESCALATION_INITIATED ● ESCALATION_REASON_RECORDED

	5. Clinician reviews the new information in clinical context (prior notes, risk, timeline). 6. Clinician determines that escalation is required. 7. Clinician selects “Initiate escalation” within the chart. 8. System requires the clinician to choose an escalation reason: ○ Abnormal result ○ Worsening symptoms ○ External report ○ Missed clinical risk ○ Other (with free text) 9. Clinician documents a short internal rationale (staff-visible only). 10. Clinician selects the escalation type: ● Expedite follow-up visit ● Order urgent investigation ● Refer to surgeon sooner ● Remove from CareQ and redirect **11.** System updates the patient’s pathway status (e.g., `escalated_under_review`). 12. System automatically creates appropriate internal tasks (e.g., booking, follow-up, admin outreach). 13. If patient contact is required, staff initiate a phone call (not messaging) to inform patient. 14. System logs that patient outreach is required (task assigned to staff). 15. Staff completes outreach and records outcome (reached / voicemail / no answer). 16. System records the escalation event in the internal case timeline. 17. Patient-facing portal updates only reflect high-level changes (e.g., status change), not internal reasoning.	● Escalation reason (structured) ● Internal clinician rationale (staff-only) ● Escalation type selected ● Pathway status before escalation ● Pathway status after escalation ● Tasks generated from escalation ● Escalation timestamp ● Clinician user ID	● ESCALATION_TASKS_CREATED ● PATIENT_OUTREACH_REQUIRED ● ESCALATION_RECORDED_IN_TIMELINE
Urgent progression to surgical consult	1. Clinician determines escalation is urgent (e.g., serious red flag, rapid deterioration, unsafe delay). **2.** System updates internal status to `urgent_progression_to_surgeon`. 3. System generates a high-priority outreach task for staff: “Call patient urgently.” 4. Assigned clinician or staff member calls the patient by phone. 5. Staff confirms patient identity (name + DOB). 6. Clinician explains verbally: ○ That concern has been identified ○ That their care is being escalated ○ That they will be moved toward surgical consultation sooner 7. Clinician answers immediate safety questions within scope. 8. Staff documents that the patient was reached and informed. 9. System updates patient-facing portal status to a patient-safe version, such as: ○ “Your care plan has been updated and your case is being prioritized for next steps.” 10. System sends SMS/email notification (no PHI): ● “An important update has been made to your CareQ plan. Please log in to view.” 11. Patient logs into portal and sees updated status and next step (e.g., “You will be contacted by the surgical clinic”).	1. Patient ID 2. Case ID **3.** Progression type = `urgent` 4. Date/time escalation decision made 5. Outreach task ID 6. Call attempt timestamps 7. Call outcome (reached / voicemail / no answer) 8. Confirmation that patient was informed (yes/no) 9. Patient-facing status after update 10. Notification sent timestamp	● URGENT_PROGRESSION_TRIGGERED ● HIGH_PRIORITY_OUTREACH_TASK_CREATED ● PATIENT_CALLED_FOR_ESCALATION ● PATIENT_REACHED ● PATIENT_NOT_REACHED ● PATIENT_STATUS_UPDATED ● ESCALATION_NOTIFICATION_SENT

	12. If patient cannot be reached by phone: ● Staff attempts repeat calls according to policy ● Outreach attempts are documented ● Portal still updates to indicate change in status		
Routine progression to surgical consult	1. Clinician determines patient is ready for progression to surgical consult as part of routine pathway. **2.** System updates internal status to `routine_progression_to_surgeon`. 3. System updates patient-facing portal status to something like: ○ "You are now moving to the next stage of your care pathway." 4. System publishes a clear patient-safe explanation in portal: ○ "Your information has been sent to the surgical clinic." ○ "You can expect contact from their booking team when they are ready." 5. System sends SMS/email notification (no PHI): ○ "Your CareQ status has been updated. Please log in to view." 6. Patient logs into portal. 7. Patient views updated status, timeline, and next-step explanation. 8. No immediate phone call is required unless patient has questions. 9. Patient remains in read-only surveillance until external clinic takes over.	1. Patient ID 2. Case ID **3.** Progression type = `routine` 4. Progression decision timestamp 5. Patient-facing status text 6. Timeline event created (e.g., "Progressed to surgical consult") 7. Notification sent timestamp 8. Patient viewed status timestamp (optional)	● ROUTINE_PROGRESSION_TRIGGERED ● PATIENT_STATUS_UPDATED ● TIMELINE_EVENT_CREATED ● PROGRESSION_NOTIFICATION_SENT ● STATUS_VIEWED_BY_PATIENT

CAREQ GENERAL CLERK

Workflow	Steps
Handle inbound patient calls	1. Clerk answers incoming call. 2. Clerk introduces themselves and CareQ. 3. Clerk verifies patient identity (name + DOB). 4. Clerk asks how they can help. 5. Clerk listens and identifies the request type (tech, scheduling, confusion, complaint, etc.). 6. Clerk provides non-clinical assistance if within scope. 7. If resolved, clerk confirms the patient understands next steps. 8. If not resolved, clerk routes the issue to the appropriate person. 9. Clerk thanks patient and ends call. 10. Clerk logs the interaction.
Provide technical support	1. Patient describes a technical issue (login, email, link, Zoom, page not loading, etc.). 2. Clerk confirms patient identity. 3. Clerk identifies the type of issue (password, missing email, broken link, device confusion).

	4. Clerk walks the patient through simple troubleshooting (e.g., check inbox, try different browser). 5. Clerk resends links, invites, or reset emails if appropriate. 6. Clerk confirms whether the issue is resolved. 7. If issue persists and seems like a system problem, clerk flags it for ops/tech support. 8. Clerk explains next steps to patient. 9. Clerk logs the issue and outcome.
Help with scheduling	1. Patient asks about booking, confirming, cancelling, or rescheduling. 2. Clerk verifies patient identity. 3. Clerk checks patient's appointment or task status in the system. 4. Clerk explains what the patient should do in the portal (e.g., where to click). 5. If needed, clerk walks patient through the steps verbally. 6. If the patient is confused or stuck, clerk may trigger resend of booking links or confirmations. 7. If scheduling appears broken (no slots, system issue), clerk flags to virtual clerk or ops. 8. Clerk confirms patient understands what will happen next. 9. Clerk logs the interaction.
Route issues to the right team member, or externally	1. Clerk identifies that the issue is outside their scope. 2. Clerk determines appropriate destination: ○ Intake clerk (referral/onboarding issue) ○ Virtual clerk (visit logistics issue) ○ Nurse/physician (clinical concern) ○ Ops/admin (policy, complaint, system issue) ○ External clinic (if appropriate) 3. Clerk summarizes the issue clearly in an internal note or task. 4. Clerk forwards the issue to the appropriate queue/person. 5. Clerk informs the patient (if appropriate) that the issue has been escalated and someone will follow up. 6. Clerk logs the escalation.
Log and summarize all interactions	1. After each call or interaction, clerk opens the patient's record. 2. Clerk records: ○ Reason for contact (tech, scheduling, confusion, etc.) ○ Short summary of what happened ○ Outcome (resolved, escalated, pending) 3. Clerk tags the interaction appropriately. 4. Clerk saves the note. 5. Other staff can see the context for future interactions.

CAREQ INTAKE CLERK

Workflow	Steps
Process new or next patients on the AHKC waitlist	1. Clerk opens the incoming queue from AHKC. 2. System shows the next unprocessed patient/referral. 3. Clerk opens the referral record. 4. Clerk reviews the attached documents (referral, imaging report, notes). 5. Clerk confirms this referral has not already been processed. 6. Clerk proceeds to data entry and validation.
Complete initial data entry	1. System displays OCR-extracted fields alongside the original document. 2. Clerk reviews patient demographics (name, DOB, contact info). 3. Clerk corrects any OCR errors. 4. Clerk fills in missing visible fields from the document. 5. Clerk goes to Alberta Netcare to pull any additional information. 6. Clerk confirms referring clinic/provider. 7. Clerk confirms reason for referral / condition. 8. Clerk saves the structured record. 9. Case becomes ready for eligibility review.
Determine eligibility for CareQ	1. Clerk reviews eligibility checklist. 2. Clerk confirms condition is appropriate for CareQ pathway. 3. Clerk confirms referral is not a duplicate. 4. Clerk confirms minimum information is present. 5. Clerk classifies the referral as: ○ Eligible ○ Ineligible ○ Requires escalation 6. Clerk selects the appropriate path.
Send ineligible patients back to AHKC	1. Clerk selects “Return to AHKC.” 2. Clerk selects a reason from a list (wrong pathway, duplicate, insufficient info, etc.). 3. Clerk adds a short free-text explanation if needed. 4. Clerk submits the return. 5. System marks the case as returned and removes it from CareQ workflow.

Escalate some urgent patient	1. Clerk identifies concerning content in referral (e.g., red flags, urgent wording). 2. Clerk selects “Escalate for clinical review.” 3. Clerk adds a short note explaining the concern. 4. System routes the case to a nurse/physician review queue. 5. Clerk does not proceed with onboarding until reviewed.
Onboard eligible patients to CareQ	1. Clerk calls the patient. 2. Clerk confirms identity (name + DOB). 3. Clerk explains what CareQ is and what the patient can expect. 4. Clerk confirms email address and phone number. 5. Clerk confirms communication preferences. 6. Clerk answers basic process questions (not clinical). 7. Clerk sends registration invite by email/SMS. 8. Clerk marks patient as “Invited to CareQ.”
Monitor onboarding funnel	1. Clerk opens onboarding dashboard. 2. Clerk sees list grouped by status: ○ Invited but not registered ○ Registered but intake not complete ○ Intake complete ○ Ready for visit 3. Clerk reviews stuck or delayed cases. 4. Clerk may resend invite or contact patient if appropriate. 5. Clerk flags unusual delays to ops if needed. 6. Clerk documents outcome internally if required.

CAREQ VIRTUAL CLERK

Workflow	Steps
Monitor daily clinic schedule	1. Clerk logs into the CareQ staff dashboard. 2. Clerk opens the **Today’s Schedule** view. 3. Clerk reviews all appointments for the day (time, patient, modality). 4. Clerk notes which visits are: ○ Confirmed ○ Unconfirmed ○ Cancelled ○ Recently rescheduled

	5. Clerk scans for obvious issues (double-booking, missing staff, unusual gaps). 6. Clerk flags any scheduling concerns to ops if needed.
Pre-visit readiness review	1. Clerk opens the list of upcoming visits (today and next 1–2 days). 2. Clerk checks for each patient: 　○ Registration completed 　○ Intake questionnaire completed 　○ Pre-visit history completed 　○ Appointment confirmed 3. Clerk identifies patients who are not ready (e.g., pre-visit incomplete). 4. Clerk may trigger resend of reminders 5. Clerk mentally flags patients who may need extra help joining.
Confirmation	1. Clerk reviews list of patients with pending confirmation. 2. Clerk verifies whether the system reminders have gone out as expected. 3. If a patient has not confirmed close to visit time, clerk prepares to expect possible issues (late, confusion, no-show). Clerk will call patient 4. Clerk remains aware of which patients may require real-time follow-up.
Prepare patients, joining first	1. Clerk joins the Zoom room a few minutes before the scheduled visit. 2. Clerk admits the patient when they arrive. 3. Clerk greets the patient and confirms identity (name + DOB). 4. Clerk confirms patient can hear and be heard. 5. Clerk briefly explains the flow ("You'll first speak with the nurse, then the physician"). 6. Clerk confirms the patient is ready. 7. Clerk hands the visit over to the nurse. 8. Clerk remains available in case issues arise during the visit.
Handle late arrivals / shows	1. Clerk monitors whether patient joins at scheduled time. 2. If patient does not join, clerk waits for a short grace period. 3. Clerk attempts to call the patient by phone. 4. If patient answers: 　○ Clerk helps them join Zoom, or 　○ Clerk converts visit to VOIP if needed. 5. If patient does not answer: 　○ Clerk marks visit as potential no-show. 6. Clerk updates appointment status accordingly. 7. Clerk ensures downstream workflow is triggered (reschedule, no-show policy, etc.).

Manage tech issues and conversion to VOIP	1. Clerk identifies technical issue (patient reports problem or clerk observes issue).
	2. Clerk attempts simple fixes:
	    ○ Ask patient to refresh
	    ○ Rejoin link
	    ○ Check mute/audio
	3. If issue persists, clerk informs patient that visit will continue by phone.
	4. Clerk initiates VOIP call to patient.
	5. Clerk confirms patient identity again.
	6. Clerk updates appointment modality to VOIP.
	7. Clerk ensures nurse/physician can proceed without disruption.
	8. Clerk documents that visit was converted due to technical issue.

Review documentation and ensure tasks are assigned	1. After a visit, clerk reviews the appointment status. 2. Clerk confirms visit is marked as completed, no-show, or cancelled appropriately. 3. Clerk checks that the visit generated follow-up actions (e.g., tasks, follow-up prompts). 4. If something appears missing (e.g., visit still open, no outcome recorded), clerk flags to nurse/physician or ops. 5. Clerk ensures no visits remain in an ambiguous state at end of day.

Participate in daily huddles	1. Clerk attends daily team huddle (if scheduled).
	2. Clerk shares:
	    ○ Any scheduling issues
	    ○ Repeated technical problems
	    ○ Patterns of no-shows
	    ○ Patients frequently struggling to join
	3. Clerk flags operational bottlenecks to ops.
	4. Clerk aligns with rest of team on any adjustments for the day.

CAREQ VIRTUAL NURSE

Workflow	Steps
Pre-visit chart review	1. Nurse logs into CareQ staff dashboard. 2. Nurse opens the list of today’s (or upcoming) patients. 3. Nurse opens the patient chart. 4. Nurse reviews intake questionnaire. 5. Nurse reviews pre-visit history.

	6. Nurse reviews previous visit summaries (if any). 7. Nurse reviews recent results (labs, imaging, reports). 8. Nurse notes missing information or inconsistencies. 9. Nurse mentally prepares areas to clarify during the visit.
Structured data review	1. Nurse joins the visit after the virtual clerk has oriented the patient. 2. Nurse introduces themselves and confirms patient identity (name + DOB). 3. Nurse explains they will ask structured questions before physician joins. 4. Nurse reviews symptoms with the patient. 5. Nurse clarifies functional impact (e.g., walking, sleep, daily activities). 6. Nurse reviews and updates medication list. 7. Nurse reviews and updates allergies. 8. Nurse confirms relevant past history. 9. Nurse confirms prior imaging/tests and timing. 10. Nurse fills in any missing fields in the structured template. 11. Nurse confirms understanding with patient before moving on.
Screen for red flags	1. During intake, nurse asks specific red-flag questions (based on template). 2. Nurse listens for concerning statements or inconsistencies. 3. Nurse identifies if any red flags are present (e.g., neurological symptoms, inability to weight bear, severe deterioration, systemic symptoms). 4. If red flags are present, nurse flags the chart as “urgent concern.” 5. Nurse prepares to highlight the concern clearly to the physician. 6. Nurse does not provide clinical judgment to the patient beyond reassurance of escalation.
Hand off to physician	1. Nurse completes the structured intake. 2. Nurse summarizes key points mentally (or in structured handoff field). 3. Nurse identifies: ○ Major changes since last visit ○ Key symptoms ○ Any red flags ○ Any uncertainty 4. Nurse informs the physician that intake is complete. 5. Nurse verbally highlights important points to the physician. 6. Nurse stays available if physician asks clarifying questions.
Ensure documentation and tasks are complete	1. After physician completes the visit, nurse reviews the chart. 2. Nurse confirms nurse intake is saved and complete.

	3. Nurse confirms physician note is saved. 4. Nurse confirms that follow-up actions exist (e.g., tasks generated, follow-up plan recorded). 5. If something is missing (note incomplete, visit still open, no plan recorded), nurse flags it to the physician. 6. Nurse ensures no visit remains in an unfinished state.
Participate in huddles	1. Nurse joins scheduled daily huddle (if applicable). 2. Nurse shares: ○ Patients with concerning symptoms ○ Repeated gaps in intake ○ Process issues noticed during visits 3. Nurse listens to updates from other team members. 4. Nurse aligns with any workflow changes for the day.

CAREQ VIRTUAL PHYSICIAN

Workflow	Steps
Pre-visit chart review	1. Physician logs into CareQ staff dashboard. 2. Physician opens today's patient list. 3. Physician opens the patient chart. 4. Physician reviews intake questionnaire. 5. Physician reviews pre-visit history. 6. Physician reviews recent results (labs, imaging, reports). 7. Physician reviews previous visit summaries (if applicable). 8. Physician reviews longitudinal timeline and trajectory. 9. Physician prepares key questions or areas to clarify.
Review the nurse intake	1. Physician opens the completed nurse intake section. 2. Physician reviews structured responses. 3. Physician reviews medication list and allergies. 4. Physician reviews functional impact and symptom severity. 5. Physician reviews nurse comments or highlights. 6. Physician identifies gaps, inconsistencies, or points needing clarification.

Assess for any red flags	1. Physician reviews any red flags flagged by the nurse. 2. Physician reviews red-flag responses in structured template. 3. Physician asks follow-up questions during the encounter if needed. 4. Physician determines whether red flags are: ○ Absent ○ Present but stable ○ Present and concerning 5. Physician mentally prepares escalation if warranted.
Interpret risk score / waitlist category	1. Physician reviews system-generated risk score. 2. Physician reviews current waitlist category. 3. Physician compares risk score with clinical presentation. 4. Physician determines whether the category is appropriate. 5. Physician confirms or adjusts the waitlist category if needed. 6. Physician considers implications for timing and urgency.
Create or update care plan, confirm any tasks	1. Physician discusses assessment with the patient. 2. Physician determines clinical plan (e.g., follow-up interval, investigations, referrals, education). 3. Physician selects structured plan elements in the platform. 4. Physician confirms which downstream tasks should be created. 5. Physician confirms whether follow-up is routine or sooner. 6. Physician confirms that plan aligns with patient understanding.
Generate structured outputs, confirm patient interest	1. Physician completes structured visit fields (risk, category, plan, disposition). 2. Physician selects appropriate structured outputs (e.g., continue pathway, escalate, discharge). 3. Physician indicates whether patient wants a written visit summary. 4. Physician confirms whether documentation should be released to patient after 24 hours. 5. System prepares downstream actions based on these selections.
Escalating concerning visits	1. Physician identifies need for escalation (clinical deterioration, concerning result, unsafe delay). 2. Physician selects escalation type (urgent vs routine). 3. Physician adds brief clinical rationale. 4. Physician confirms escalation action. 5. System routes case to appropriate pathway (e.g., surgeon, urgent review queue). 6. Physician explains escalation to the patient during the encounter.

	7. In most cases, physician will call the surgeon later that day.
Review and finalize any clinical documentation	1. Physician reviews the AI-generated or templated visit note. 2. Physician edits wording as needed for accuracy. 3. Physician ensures assessment and plan are clearly documented. 4. Physician signs or finalizes the note. 5. Physician confirms that the visit status is marked complete.
Lead any focused patient reviews, as needed	1. Physician is notified of a complex or concerning case. 2. Physician opens the full chart and history. 3. Physician reviews longitudinal trajectory and prior decisions. 4. Physician discusses the case with nurse or team if needed. 5. Physician determines updated plan or disposition. 6. Physician documents the decision clearly in the chart. 7. Physician triggers any required escalation or plan changes.

CAREQ WAITLIST NURSE

Workflow	Steps
Sending e-consults	1. Clerk opens the list of patients requiring e-consult. 2. Clerk opens the patient chart. 3. Clerk reviews the physician’s documented plan and rationale. 4. Clerk opens the e-consult template. 5. Clerk confirms the receiving specialist / clinic. 6. Clerk ensures required clinical information is included (history, reason, relevant findings). 7. Clerk attaches any relevant documents (notes, reports, imaging summaries). 8. Clerk submits the e-consult through the designated channel. 9. Clerk marks the e-consult as “Sent” in the system.
Closing loop on e-consults	1. Clerk opens the e-consult tracking list. 2. Clerk reviews which e-consults are still outstanding. 3. Clerk checks for incoming responses. 4. When a response arrives, clerk opens and reviews the content. 5. Clerk attaches the response to the patient record. 6. Clerk routes the response to the nurse/physician review queue. 7. Clerk marks the e-consult status as “Response received.” 8. If no response after expected timeframe, clerk flags as overdue.

	9. Clerk may resend or escalate overdue e-consults per process.
Send investigations	1. Clerk opens the list of investigation tasks generated by physicians. 2. Clerk opens the patient chart. 3. Clerk confirms which tests are required. 4. Clerk selects the appropriate lab or imaging destination. 5. Clerk completes the investigation requisition. 6. Clerk ensures patient demographics are correct. 7. Clerk sends the requisition via the designated channel. 8. Clerk marks the investigation as “Sent.” 9. Clerk ensures the patient has been informed (if required by workflow).
Closing loop on investigations	1. Clerk opens the investigation tracking dashboard. 2. Clerk reviews which investigations are still outstanding. 3. Clerk monitors for incoming lab or imaging results. 4. When a result arrives, clerk opens the report. 5. Clerk confirms the result belongs to the correct patient. 6. Clerk attaches the result to the patient chart. 7. Clerk routes the result to the nurse/physician review queue. 8. Clerk updates investigation status to “Result received.” 9. If result is overdue beyond expected timeframe, clerk flags as overdue. 10. Clerk escalates persistently overdue or concerning cases to ops/clinical review.

CAREQ WAITLIST PHYSICIAN

Workflow	Steps
Sending e-consults	1. Physician opens list of patients requiring e-consult. 2. Physician opens the patient chart. 3. Physician reviews recent nurse intake, notes, and results. 4. Physician determines that specialist input is clinically indicated. 5. Physician selects the receiving specialist / clinic. 6. Physician completes the structured e-consult content (question, context, urgency). 7. Physician confirms attachments are appropriate (notes, results, imaging summaries). 8. Physician submits the e-consult. 9. System marks e-consult as “Sent.”
Closing loop on e-consults	1. Physician opens e-consult tracking list. 2. Physician reviews which e-consults have responses available.

	3. Physician opens and reads the specialist response. 4. Physician interprets recommendations in clinical context. 5. Physician determines whether the plan should change. 6. Physician updates the care plan if needed. 7. Physician documents interpretation in the chart. 8. Physician confirms next steps (follow-up, escalation, reassurance, etc.). 9. System marks e-consult as “Reviewed.”
Send investigations	1. Physician reviews patient chart and clinical trajectory. 2. Physician determines that new investigations are clinically indicated. 3. Physician selects appropriate investigations (labs, imaging). 4. Physician enters or confirms investigation orders in the system. 5. System generates tasks for clerk to send requisitions. 6. Physician documents rationale briefly in the chart. 7. System marks investigation as “Ordered.”
Closing loop on investigations	1. Physician opens the list of new results requiring review. 2. Physician opens each result. 3. Physician interprets results in clinical context. 4. Physician determines whether results are: ○ Reassuring ○ Require monitoring ○ Require action 5. Physician updates care plan if needed. 6. Physician documents interpretation and plan. 7. Physician confirms whether patient communication is required. 8. System marks result as “Reviewed.”
Escalating concerning results	1. Physician identifies a concerning result or deterioration. 2. Physician determines urgency (same day, urgent, semi-urgent). 3. Physician contacts the patient by phone. 4. Physician explains the finding and its implications. 5. Physician assesses symptoms and safety on the call. 6. Physician provides clear next steps to the patient. 7. Physician determines whether escalation to surgeon is required. 8. If escalation is required: ○ Physician prepares a concise clinical summary. ○ Physician contacts the surgeon or sends urgent referral/e-consult. 9. Physician documents the escalation rationale and actions taken. 10. System marks the case as “Escalated.”

CAREQ ADMIN

Workflow	Steps

Live operations monitoring	1. Ops opens the live operations dashboard.
	2. Ops views all active clinic rooms.
	3. Ops sees which patients are:
	    ○ Waiting
	    ○ In visit
	    ○ Disconnected
	    ○ No-show
	4. Ops sees which staff are:
	    ○ Logged in
	    ○ In a room
	    ○ Idle
	    ○ Offline
	5. Ops identifies visits running behind schedule.
	6. Ops checks for patients waiting too long.
	7. Ops intervenes if needed (message staff, reassign flow, step in).
	8. Ops continuously monitors throughout clinic hours.

Performance monitoring	1. Ops opens performance dashboard.
	2. Ops reviews daily metrics:
	    ○ Visits scheduled
	    ○ Visits completed
	    ○ No-shows
	    ○ Cancellations
	3. Ops reviews operational metrics:
	    ○ Time per visit
	    ○ Time waiting before join
	    ○ Onboarding completion rate
	    ○ Backlogs (e-consults, results, tasks)
	4. Ops reviews trend views (weekly / monthly).
	5. Ops identifies concerning patterns (rising no-shows, delays, bottlenecks).
	6. Ops flags issues to team or adjusts workflows as needed.

User management	1. Ops opens user management panel. 2. Ops invites new staff by email. 3. Ops assigns role (clerk, nurse, physician, etc.). 4. Ops sets access permissions based on role.

	5. Ops activates or deactivates accounts as staff join/leave. 6. Ops resets passwords if needed. 7. Ops audits user list periodically for accuracy.
Schedule and clinic control	1. Ops opens the clinic schedule view. 2. Ops views upcoming clinic sessions and staffing. 3. If a clinician is unavailable, Ops cancels or pauses that clinic session. 4. Ops reassigns patients to other sessions if appropriate. 5. Ops blocks time slots when needed (e.g., emergencies, system issues). 6. Ops can add ad hoc sessions if capacity is needed. 7. Ops confirms patients are notified by system when schedule changes occur. 8. Ops monitors downstream impact (reschedules, confusion, overload).
Admin configuration	1. Ops opens configuration settings panel. 2. Ops reviews current parameters (e.g., reminder timing, no-show thresholds). 3. Ops adjusts settings within approved policy limits: ○ Reminder schedules ○ Visit durations ○ Confirmation windows ○ Escalation thresholds 4. Ops saves configuration changes. 5. Ops monitors impact of changes over subsequent days. 6. Ops reverts or fine-tunes if negative effects appear.
Quality assurance / quality control	1. Ops opens list of recently completed visits. 2. Ops randomly selects charts for review. 3. Ops checks for: ○ Completed documentation ○ Tasks properly generated ○ Escalations handled correctly ○ Workflow steps followed 4. Ops identifies any deviations or gaps. 5. Ops provides feedback to relevant staff. 6. Ops flags systemic issues for workflow adjustment. 7. Ops tracks recurring quality issues over time.
Billing	1. Ops opens billing overview dashboard. 2. Ops reviews which visits are marked as billable.

	3. Ops checks for missing or incomplete billing data. 4. Ops flags discrepancies (e.g., completed visit not billed). 5. Ops coordinates with billing consultant if external. 6. Ops reviews high-level billing summaries (volume, rejection rates). 7. Ops identifies patterns of billing issues and escalates internally.

AHKC WAITLIST SURGEON

Workflow	Steps
Review and respond to e-consults	1. Surgeon opens the e-consult queue. 2. Surgeon selects the next e-consult. 3. Surgeon reviews the clinical summary provided by the CareQ physician. 4. Surgeon reviews relevant attachments (notes, imaging reports, results). 5. Surgeon determines whether the case is: ○ Surgically appropriate ○ Not surgically appropriate ○ Requires more information 6. Surgeon determines urgency (routine vs expedited vs urgent). 7. Surgeon enters a brief response with recommendations (e.g., accept, defer, request imaging, reassess in X months). 8. Surgeon submits the response. 9. System marks the e-consult as “Completed” and routes back to CareQ.
Handle urgent clinical escalations	1. Surgeon receives an urgent call from CareQ physician. 2. Surgeon listens to a concise case summary. 3. Surgeon asks clarifying questions as needed. 4. Surgeon reviews any relevant data shared verbally or on-screen. 5. Surgeon determines level of concern and urgency. 6. Surgeon provides clear guidance (e.g., urgent in-person consult, expedited pathway, reassurance with monitoring). 7. Surgeon confirms next steps with the CareQ physician. 8. Surgeon expects the CareQ physician to document the interaction.

AHKC BOOKING CLERK

Workflow	Steps

Pull booking-ready patients from CareQ	1. Clerk logs into CareQ. 2. Clerk opens the **“Ready for Surgical Booking”** view. 3. System shows patients flagged as: ○ Cleared for consult ○ Up-to-date ○ Not escalated elsewhere 4. Clerk sorts/filter by surgeon, urgency, or date as needed. 5. Clerk selects the next patients to book based on clinic capacity.
Book patients into surgical consults	1. Clerk opens AHKC booking system. 2. Clerk identifies available consult slots. 3. Clerk selects next appropriate patient from CareQ list. 4. Clerk contacts patient (phone) to offer appointment time. 5. Clerk confirms date/time with patient. 6. Clerk completes booking in AHKC system. 7. Clerk confirms appointment verbally with patient.
Update CareQ when booking is completed	1. Clerk returns to CareQ. 2. Clerk opens the patient record. 3. Clerk marks status as **“Booked for surgical consult.”** 4. Clerk enters date/time of appointment (if field exists). 5. System removes patient from active CareQ surveillance. 6. Patient now appears as “Transitioned to surgical consult.”

AHKC ADMIN

Workflow	Steps
Monitor waitlist movement	1. Admin opens AHKC / CareQ waitlist dashboard. 2. Admin reviews: ○ Total waitlist volume ○ New additions ○ Patients moving to consult ○ Patients exiting CareQ 3. Admin reviews time-in-category (how long people sit in each tier). 4. Admin looks for stagnation, bottlenecks, or unusual patterns. 5. Admin flags concerns to leadership or ops if needed.

Monitor waitlist performance	1. Admin reviews high-level metrics: ○ Average wait time ○ Distribution by urgency ○ Variability across surgeons ○ Completion rates 2. Admin identifies inequities or inconsistencies. 3. Admin investigates root cause (capacity, process, triage). 4. Admin escalates structural issues to leadership.
Monitor escalation patterns	1. Admin opens escalation dashboard. 2. Admin reviews: ○ Number of escalations ○ Reasons for escalation ○ Outcomes of escalations 3. Admin looks for: ○ Too many escalations from same pathway ○ Delays in response ○ Patterns suggesting unsafe wait times 4. Admin flags systemic safety concerns.
Monitor patient experience	1. Admin reviews patient satisfaction summaries. 2. Admin reviews complaints and themes. 3. Admin reviews no-show and dropout patterns. 4. Admin identifies whether communication/process is causing harm. 5. Admin recommends process improvements.